Lutetium-177 oxodotreotide (Lutathera / peptide receptor radionuclide therapy (PRRT))

National Cancer Control Programme (NCCP) 00642 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending

PRACTICAL TREATMENT GUIDE

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Before admission

Confirm somatostatin receptor (SSTR) imaging, appointments, recent bloods, hormone medicines and pregnancy precautions. Tell the team about kidney/heart problems, incontinence, a catheter, childcare or close contact with pregnant people; these may change the practical plan.

STEP 2

Treatment day

The standard NCCP activity is 7.4 GBq by intravenous infusion, usually every eight weeks for four administrations; the specialist may alter, delay or stop treatment. Lutathera runs over approximately 30–40 minutes. A lysine/arginine amino-acid infusion starts 30 minutes beforehand and continues for four hours. An anti-sickness medicine is given at least 30 minutes before the amino acids. The first visit generally includes an overnight stay; later treatments may be day case or overnight. The service confirms your plan.

STEP 3

somatostatin analogue (SSA) injections: stop and restart

The neuroendocrine tumour (NET) and nuclear-medicine teams must coordinate the injection plan. Do not translate “about four weeks” into your own date: NCCP specifies 30 days for long-acting SSA and 24 hours for short-acting SSA before PRRT. Restart timing follows the written service plan; it can differ with day-case arrangements.

STEP 4

At home after treatment

Before discharge, obtain a paper plan with the end date for each restriction on contact, sleeping, work, travel and body-fluid handling. First-week instructions include daily bathing, careful handwashing, sitting to pass urine and separately washing body-fluid-soiled laundry. Ask for specific advice about pads, incontinence or a catheter. Examples include sleeping separately from a partner for 14 days and from pregnant people or children for three weeks; restrictions differ by contact and circumstance. Your medical physicist sets your personal instructions. Tell them before discharge if you cannot follow the plan at home.

STEP 5

Travel and other healthcare

Carry the discharge letter and the official radiation-treatment card for the period the medical physicist specifies. They identify the treatment and date and help with travel, radiation detectors and other healthcare. The SACTCheck wallet card is an additional record and does not replace them. Tell healthcare staff about the treatment; radiation precautions must never delay urgent care.

STEP 6

Pregnancy, breastfeeding and fertility

Lutathera can harm an unborn child and may affect fertility. Discuss fertility preservation before treatment. Use effective contraception during treatment and for seven months after the last dose for patients who could become pregnant; men with partners who could become pregnant need contraception during treatment and for four months afterwards. Breastfeeding must stop before treatment; follow specialist advice.

STEP 7

If a visit is delayed

Low blood counts, kidney or liver changes, other side effects or a change in the cancer may lead the specialist to postpone, reduce or stop treatment. A four-stamp record shows planned administrations, not medical clearance. A dose does not automatically become suitable at the maximum interval. Keep the blood-test and symptom plan while waiting for the team's decision.

STEP 8

Contacts and paperwork

Write the actual NET, PRRT nursing and radiation-protection contacts on the paper plan. Example numbers are not clinical contact lines. Keep the written precautions and urgent hormone-symptom instructions with the discharge letter.

STEP 9

Your specialist team's scan appointments

Radiation-assessment scans measure where the treatment went and the radiation delivered to organs. They answer a different question from computed tomography (CT) or magnetic resonance imaging (MRI) scans of tumour response. The detailed treatment information describes three radiation scans on separate days in the first week after the first infusion, starting the next day, and one after later infusions; response CT or MRI is described between the second and third infusions and after the course. Confirm your actual bookings with the team: these are service arrangements, not a universal timetable for every neuroendocrine tumour.

STEP 10

Extra blood tests after the course

Blood counts, kidney and liver tests continue every four weeks for at least three months after the last administration, then every three months for the following year. Your written discharge plan may also include two-weekly tests for the first eight weeks after the last infusion. The team confirms those extra early appointments and who reviews the results.

Sources: NCCP 00642 v2 · Lutathera regimen. Source reconciled; independent clinical, pharmacy and patient/PPI review pending.